

GUIDELINES

S2k guideline for treatment of cutaneous lupus erythematosus – guided by the European Dermatology Forum (EDF) in cooperation with the European Academy of Dermatology and Venereology (EADV)

Abstract

Cutaneous lupus erythematosus (CLE) is a rare inflammatory autoimmune disease with heterogeneous clinical manifestations. To date, no therapeutic agents have been licensed specifically for patients with this disease entity, and topical and systemic drugs are mostly used 'off-label'. The aim of the present guideline was to achieve a broad consensus on treatment strategies for patients with CLE by a European subcommittee, guided by the European Dermatology Forum (EDF) and supported by the European Academy of Dermatology and Venereology (EADV). In total, 16 European participants were included in this project and agreed on all recommendations.

Topical corticosteroids remain the mainstay of treatment for localized CLE, and further topical agents, such as calcineurin inhibitors, are listed as alternative first-line or second-line topical therapeutic options. Antimalarials are recommended as first-line and long-term systemic treatment in all CLE patients with severe and/or widespread skin lesions, particularly in patients with a high risk of scarring and/or the development of systemic disease. In addition to antimalarials, systemic corticosteroids are recommended as first-line treatment in highly active and/or severe CLE.

Second- and third-line systemic treatments include methotrexate, retinoids, dapsone, and mycophenolate mofetil or mycophenolic acid, respectively. Thalidomide should only be used in selected therapy-refractory CLE patients, preferably in addition to antimalarials. Several new therapeutic options, such as

B-cell- or interferon α -targeted agents, need to be further evaluated in clinical trials to assess their efficacy and safety in the treatment of patients with CLE.

Introduction

Lupus erythematosus is an inflammatory autoimmune disease that may involve severe systemic organ manifestations (systemic lupus erythematosus, SLE), or may be limited to the skin (cutaneous lupus erythematosus, CLE). Based on clinical features, histological findings, and serological abnormalities, four subtypes of CLE are recognized:

- Acute CLE (ACLE)
- Subacute CLE (SCLE)
- Chronic CLE (CCLE), including discoid lupus erythematosus (DLE), chilblain lupus erythematosus (CHLE), and lupus erythematosus panniculitis (LEP)
- Intermittent CLE (ICLE), also known as lupus erythematosus tumidus (LET)

To date, no drugs have been approved specifically for the treatment of CLE, although several therapies are approved for SLE, including the monoclonal antibody belimumab. Therefore, both topical and systemic treatments for CLE are predominantly used off-label and are rarely supported by high-level evidence from randomized controlled trials.

These guidelines aim to provide consensus-based recommendations for the treatment of CLE. Due to the heterogeneity of skin manifestations, therapeutic approaches must be individualized and initiated by specialists experienced in managing this condition.

The target audience for these treatment guidelines includes dermatologists and other specialists involved in the care of CLE patients, such as rheumatologists and nephrologists. Separate guidelines focusing on diagnosis and monitoring of CLE, intended for a broader audience including general practitioners, are under development and will be published independently.

Methods

Given the absence of standardized treatment protocols, this project aimed to develop European consensus guidelines for the management of CLE, supported by the European Dermatology Forum (EDF) and the European Academy of Dermatology and Venereology (EADV).

Professor Annegret Kuhn, chair of the guideline subcommittee, in collaboration with experts from the European Society of Cutaneous Lupus Erythematosus (EUSCLE), selected members of the subcommittee in 2013. One expert was invited from each participating center or country, resulting in a total of 16 participants. A consensus-driven approach was used to develop the recommendations presented in this guideline.